



HIGHER HEALTH

Higher Education & Training: Health, Wellness and Development Centre

STRATEGIC PLAN 2026-2031

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Introduction

HIGHER HEALTH has shown agility and has grown remarkably in the decade since it became an independent non-profit organisation. It is now set for a bold transition from a health and wellness organisation to a provider of wraparound support that responds holistically to the realities of 21st century students in the post-school education and training (PSET) sector.

The organisation's expansion has been driven by an ambition to serve the PSET sector in its entirety while its new identity is a response to the profound shift in students' needs and prospects in a fast-changing world, driven by quantum leaps in technology.

Our intention, embodied in HIGHER HEALTH 3.0, is to build a fully integrated ecosystem of support services, extending beyond traditional health and psychosocial services, to address the vulnerabilities and meet the developmental needs and aspirations of students.

We aim also to expand our reach to offer community-based programmes for young people who are not in employment, education, and training (NEET) to enhance their prospects of earning a livelihood.

HIGHER HEALTH recognises that today's students face bewildering challenges. Many are from disadvantaged communities and continue to contend with the stresses of poverty as they study. Yet academic success, while demanding discipline and sacrifice, does not guarantee students a better life. The ground is constantly shifting under their feet as technological change reshapes economies and career options. Change is the constant reality of today's students – and we need to provide them with tools to ride the tide of change.

The Strategic Plan 3.0 embodies HIGHER HEALTH's enterprising spirit and enables us to transform of our programmes to suit our changeable climate. The plan stays firmly anchored in evidence on the particular needs and challenges of our current generation of students, the requirements of PSET institutions, and South Africa's thirst for a workforce with knowledge and skills that will enable economic growth.

The ecosystem of wraparound support

HIGHER HEALTH has given the concept of wraparound support a dynamic developmental spin. We have taken an approach pioneered to tackle complex health and psychosocial issues in therapeutic settings and applied it to the equally complex challenges that young people face as they strike out into the world of study and work.

The essential features of wraparound support are:

- The provision of a **wide range** of aid, relevant to the people it serves.
- The **integration** of these interventions, so that they are mutually reinforcing.
- **Individualisation** of support by combining programme elements following each person's needs.

Wraparound programmes have the potential to empower students and enhance their independence because they tackle a whole number of vulnerabilities and experiential gaps simultaneously.

Essential elements of wraparound support for South African PSET students are:

- Development of life skills to transform vulnerabilities into resilience.
- Psychosocial care to respond to the stress and anxiety that students often experience.
- Primary health care to meet the physical health challenges and priorities of young adults.
- Acquisition of soft skills – such as teamwork, communication, and time management – that are crucial to academic success and work readiness.
- Cultivation of civic responsibility and community mindedness to create an inclusive culture.

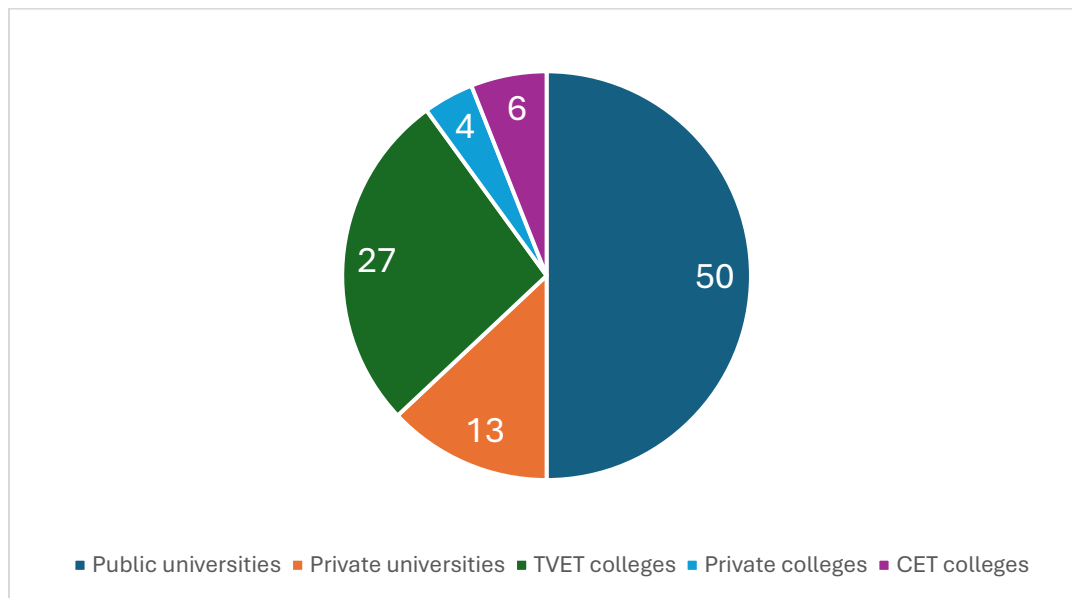
The National Student Financial Aid Scheme (NSFAS) plays an important complementary role to HIGHER HEALTH by funding the studies and subsistence of hundreds of thousands of young people from low-income families every year.

The reality that informs our strategic choices

The world we work in

The PSET sector catered to 2.12 million students in 2023, according to the most recent statistical report of the Department of Higher Education and Training (DHET).

Figure 1: Distribution of students by category of PSET institution (%)



Note: The figure for private colleges only includes students enrolled at DHET-registered colleges and excludes those accredited solely by the Quality Council for Trades and Occupations (QCTO).

Currently, HIGHER HEALTH supports public sector institutions – 26 universities, 50 TVET colleges and nine CET colleges – which enrolled 83% of all students recorded by the DHET.

- Nearly half these students were NSFAS beneficiaries.
- Female students significantly outnumbered male students.

Private universities and colleges had a combined enrolment of about 167 000 in 2023 and workplace-based training under sector education and training authorities (SETAs) catered to 148 960 young people. Both settings present opportunities for HIGHER HEALTH to expand its model of wraparound support.

A massive 3.6 million young people aged 15 - 24 years are not in employment, education or training (NEET). Including them in some form of PSET is a challenge to the education system and HIGHER HEALTH has the potential to help meet this challenge.

What our students and youth need

HIGHER HEALTH has developed its wraparound support systems in response to documented health, psychosocial and development needs of students and other youth, which include:

Mental health challenges: The psychosocial pressures on PSET students are considerable. They have transitioned to a unfamiliar and demanding academic world. Family expectations may be high, financial stresses severe, and support systems minimal. Furthermore, adolescence and early adulthood are life stages in which serious mental health problems tend to manifest. A national survey of 29 000 public university students, published in 2022, found one in five needed psychosocial support and a small sub-group (4%) needed specialised mental health services.

Social media, mental health, and people skills: While digital communication has been a great enabler of progress, social media have a downside. Research has shown the addictive nature of

some social media platforms and their potential harm to the mental health of young people. Landmark court cases abroad have accepted this. Research also suggests that excessive reliance on digital engagement can reduce social activity and weaken people skills.

Gender equality and gender identity: Our constitutional right to equality and social movements for women's and LGBTQI rights have had implications for masculine identity. Gender-based violence (GBV) has been linked to toxic forms of masculinity. There is work to be done to overcome damaging and restrictive gender norms.

Youth unemployment: The spectre of unemployment after years of study haunts students. Four out of 10 South Africans aged 25-34 years were out of work in early 2026. Course completion is vital for students as a PSET qualification increases the chances of securing employment and work-ready youth have the edge in competing for jobs.

The continued threat of HIV and STIs: While the incidence of HIV has declined, a majority of PSET students fall into the demographic at highest risk of infection: young women. In 2022, about 7.4% of South African women were living with HIV at 18 years of age and by the age of 25 years the rate was 17.3% (Thembisa Model 4.8).

Sexual and reproductive health: Health service packages for young people include a strong focus on sexual and reproductive health (SRH). In terms of retaining woman students and boosting course completion rates, accessible contraceptive services are a priority. Contraceptive coverage for young women is lower than desired and HIGHER HEALTH strives to address barriers to uptake.

Gender-based violence (GBV) and femicide: South Africa's first national study on GBV, conducted by the Human Sciences Research Council in 2022, found one in three women aged eighteen years or older had experienced physical and/or sexual violence. Young women aged 18-24 years were at particular risk of violence perpetrated by a non-partner.

Risky alcohol and substance use: Various small studies confirm that frequent and risky use of alcohol is common among sections of the student population. Use of cannabis is also widespread. Risky consumption is often driven by cultural norms that encourage excessive drinking and ignorance about the dangers of alcohol and other substances.

Problem gambling: There is strong anecdotal evidence that online gambling on mobile platforms is out of control for a growing number of students. HIGHER HEALTH's counsellors have seen this, as have investigative journalists and other youth services. Students often turn to gambling because they are in dire financial need, but this only compounds their difficulties.

PSET sector performance

The PSET sector faces certain performance issues which HIGHER HEALTH may help address. These include:

Throughput and course completion: The percentage of university students who complete their undergraduate degrees in the three-year period has risen from 18.8% for the 2009 intake to 28.3% for the 2019 intake. But "there are still far too many students who take too long to complete their university degrees, thereby burdening the system in terms of funding and available space," the

DHET's Post-school Education and Training Monitor 2024 reported. The TVET college throughput rate for the three-year NC(V) Level 2 course was 10.8% in 2021 and it has not improved.

Access for people with disabilities: In 2021, students living with disabilities made up only 1.2% of total university enrolment, 0.8% of TVET college enrolment and 0.2% of CET college students. This is significantly lower than the proportion of people with disabilities in the general population. The DHET Monitor 2024 acknowledges “the absence of proactive interventions and lack of capacity and resources contribute to this phenomenon”.

Social inequality and academic outcomes: Research on the completion rate gap between privileged and disadvantaged students highlights the continued impact of poverty on many students, the stigma associated with poverty, the inferiority of secondary schools in townships and rural areas, and the sense among black students that the academic system is unfriendly to them. The evidence argues for better student support systems and a more inclusive institutional culture.

Workplace-ready graduates: An enduring discussion between the PSET sector and employers centres on the sector's ability to produce graduates with skills needed in the workplace. To some extent this concerns the production of graduates with high-demand technical knowledge and skills. But it also centres on soft skills such as communication, ethical decision-making, and teamwork. These uniquely human abilities are increasingly important as artificial intelligence assumes many routine functions.

Key Features of HIGHER HEALTH 3.0

This five-year strategy is a balance between continuity and innovation. The core principles that guide HIGHER HEALTH are familiar, while innovation features in three areas. These are summarised below.

Enduring principles

Some key principles lie at the heart of HIGHER HEALTH and will continue to sustain the organisation in the period ahead. They are:

Partnership: HIGHER HEALTH's work depends on strong, dynamic partnerships with universities, TVET colleges and CET colleges. We implement our programmes with and through PSET institutions. Further partnerships – with funders, the DHET and related public entities, public health authorities, non-profit organisations, and research institutions – are critical to fulfilling our mission.

Personal development: HIGHER HEALTH strives to enable young people to devise their own solutions to their life challenges through educational and skills-building interventions. The essence of our work is developmental, focused on building resilience and life skills to manage health and psychosocial challenges, generic soft skills to enhance academic and career success, and civic values to encourage students to contribute to nation building and social cohesion.

Prevention: HIGHER HEALTH's interventions focus on the promotion of physical and psychosocial health and the prevention of illness. We create student awareness of the links between health and lifestyle choices, provide them with tools for risk identification, offer preventive health products, and expand access to primary health care and psychosocial services for early diagnosis and treatment.

Peer-driven interventions: While HIGHER HEALTH and PSET institutions provide key human resources to manage and deliver our programmes, peer-to-peer activities by student volunteers are a constant and significant feature. Not only has peer engagement been shown to enhance the impact of health and social programmes, but it offers students opportunities to acquire the soft skills that are sought in the job market.

Tech-enablement: HIGHER HEALTH is committed to the proper use of technology to expand student access to its services and programmes in an efficient way. Its online formal educational programmes and social media campaigns have led the way. Telemedicine facilities will be introduced under HIGHER HEALTH 3.0 to offer students across the sector, in the remotest corner, access to consultations with medical practitioners and other health professionals.

Priorities for innovation

Three major areas of innovation are shown for the period 2026/7-2031/2:

Priority 1: Skills development for young people to enable their personal growth and contribution to social cohesion

Education – the transfer of knowledge and values, and the building of skills – is the most potent tool for individual empowerment and nation building. It has the power to transform individual lives, communities, and whole societies. HIGHER HEALTH places formal and non-formal, accredited, and non-accredited educational interventions at the centre of our strategy.

Our Civic and Soft Skills Programme offers a unique combination of enabling content for young people and we plan to make it more widely available. We intend to find mechanisms for offering the programme to youth who are currently NEET, to private colleges and universities, and employers looking to enhance the skills of young employees.

Table 1: Mechanisms to offer Higher Health Programmes

Component	Content	Delivery system
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QCTO-accredited NQF Level 5 Civic and Soft Skills Programme, including: Civic education Life skills for health and wellness Soft skills for academic success and work readiness	Core learning areas: Civic education Mental health awareness Transformative gender education Comprehensive sexuality education Disability and social inclusion Substance abuse and resilience Environmental justice Financial literacy Digital literacy and ethics of AI	Hybrid learning Full online learning programme supported by interactive in-person sessions led by HIGHER HEALTH facilitator Practical requirements designed to build soft skills: Emotional intelligence and empathy Critical thinking and problem solving Communication Confidence and resilience Goal setting and time management
Peer-to-peer learning, including: Non-accredited development programmes Extramural programmes Campus engagement activities Community-based activities	Mental health awareness Prevention of GBV Prevention of alcohol and substance abuse Gambling risk awareness Cyber wellness Leadership development Human rights and civic participation	The essence is learning through peer-facilitated engagement during: Campus-wide campaigns and dialogues Formal learning sessions Support group discussions Community outreach

Priority 2: Inclusion of vulnerable youth and support for their success

HIGHER HEALTH aims to make the PSET sector an outstanding example of inclusion and support for vulnerable young people. We aim to strengthen our existing interventions for students with substance abuse issues, the LGBTQI community on campuses, and young people with disabilities. In addition, we will raise awareness of problem gambling among students and help those with gambling concerns.

HIGHER HEALTH intends to focus strongly on the youth with disabilities, both physical disabilities and learning and neurodevelopmental disabilities. The barriers faced by young people with disabilities often deter them from considering education beyond school. Those who gain entry to PSET often experience exclusion from learning and social activities, a lack of psychosocial and learning support, and poorer prospects of employment.

Table 2: Services offered to the youth with disabilities

Component	Interventions
Identification and risk screening	Identification of vulnerable students through peer-to-peer outreach, informal educational activities, and use of self-screening tools

	Professional screening to identify special service and support needs of vulnerable students, including psychosocial services
Standard packages to accommodate learning and participation for students with disabilities	Learning support systems Assistive technologies Tutoring and learning assistance Accessibility interventions: physical and digital Academic accommodation Psychosocial support Institutional inclusion frameworks
Disability support ecosystem	Clinical services Psychosocial support Academic support Student retention service Career guidance Personal development modules Economic empowerment modules

Priority 3: Enhancing student access to primary health care through dedicated clinics and technology-enabled services

HIGHER HEALTH will continue to focus on health promotion, disease prevention, and primary health care. We envisage expanding our primary healthcare services and ensuring easy access for students regardless of where they are found. We will do this by upskilling health professionals at campus and mobile clinics, and by introducing a significant telemedicine component. We will set up a telemedicine hub, staffed by general medical practitioners and other health professionals, who can be consulted online by students. They will also provide guidance to practitioners in our clinics.

Table 3: Student access to Primary Healthcare

Component	Services	Health personnel
Primary healthcare services, including: Sexual and reproductive health HIV prevention and treatment Care following gender-based violence (GBV) Non-communicable disease management	24-hour health and wellness helpline Consultations with health professionals (on-line and in-person) Risk-screening tools Digital symptom screening and referrals Preventive health products	Doctors Nurses

Mental health and psychosocial support Academic stress Anxiety and depression Relationship issues Social isolation Trauma support GBV prevention and response Alcohol and substance abuse Compulsive gambling Digital harms	Awareness and prevention campaigns Early warning and referral systems Crisis support systems Counselling services Survivor support groups Peer support networks	Nurses Social workers Psychologists Helpline counsellors
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Organisational sustainability

HIGHER HEALTH 3.0 approaches organisational sustainability not only from the critical angle of mobilising and managing the necessary financial and human resources, but also in terms of sustaining good governance and upholding our reputation through ethical conduct and the delivery of quality programmes.

Vision, mission, and values

The vision and mission statements are complementary and should be read together.

Vision

An ecosystem of wrap-around support for the youth, including all students in the PSET sector.

Mission

To deliver integrated skills development and accessible health and psychosocial support that enable young adults to succeed academically and transition into work.

Values

Integrity:	We uphold the highest ethical standards in all that we do
Transparency:	We build trust through openness and accountability
Innovation:	We innovate continuously to remain relevant and effective
Collaboration:	We work together to build a better future
Entrepreneurial mindset:	We capitalise on opportunities for growth and sustainability

Strategic goals and objectives

Goal 1

To ensure youth in PSET and workplaces and youth who are NEET have access to quality-assured, accredited and non-accredited programmes in civic education, life skills, and broader soft skills to promote personal development, employability, active citizenship, and sustainable futures

Goal 2

To advance equitable access to quality primary health care, psychosocial services, and integrated support services for students across PSET institutions

Goal 3

To create an enabling environment that increases awareness, strengthens knowledge and promotes prevention and early intervention in respect of health, social well and environmental challenges and community-centred epidemics in the PSET sector

Goal 4

To enhance the technical capacity and preparedness of frontline and managerial staff, student leadership, and volunteers in the PSET sector through continuous training, technical support, and scientific knowledge-sharing on health risks, psychosocial matters, emerging epidemics, and other community-centred challenges

Goal 5

To foster effective sectoral coordination and collaboration across the PSET system, ensuring the sharing of best practices and joint implementation

Goal 6

To mobilise sustainable national and global financial, technical and strategic partnerships, underpinned by good governance and accountability, to strengthen holistic wraparound support

Objectives

Objectives corresponding to the six strategic goals are presented below.

Table 4: Goals and objectives

Goals	Objectives
1. To ensure youth in PSET and workplaces and youth who are NEET have access to quality-assured, accredited and non-accredited programmes in civic education, life skills, and broader soft skills to promote personal development, employability, active citizenship, and sustainable futures	Provide the youth at PSET institutions, workplaces, and youth who are NEET with access to HIGHER HEALTH's accredited civic, life skills and soft skills programmes
	Provide the youth at PSET institutions, workplaces, and youth who are NEET with access to HIGHER HEALTH's non-accredited skills programmes
2. To advance equitable access to quality primary health care, psychosocial services, and integrated support services for students across PSET institutions	Expand access to primary health care for students across PSET institutions through campus-based and digital services, and a referral support system
	Strengthen mental health and psychosocial support services to provide GBV and mental health services on campuses

	Implement comprehensive health and wellbeing wraparound support for students with disabilities
3. To create an enabling environment that increases awareness, strengthens knowledge, and promotes prevention and early intervention in relation to health, social and environmental challenges, and community-centred epidemics in the PSET sector	Implement peer-led awareness programmes across PSET institutions through coordinated campaigns, wellness days, dialogues and community engagement initiatives among staff and students
	Expand the reach and impact of health, psychosocial and developmental messaging through integrated digital platforms, social media campaigns, radio programmes, and other multi-media/technology-driven initiatives
4. To enhance the technical capacity and preparedness of frontline and managerial staff, student leadership, and volunteers in the PSET sector through continuous training, technical support, and scientific knowledge-sharing on health risks, psychosocial matters, emerging epidemics and other community-centred challenges	Establish and strengthen a sustainable peer-to-peer education and student leadership programme across PSET institutions
	Enhance the capacity and responsiveness of frontline and managerial staff within the PSET sector through continuous training, technical development, and skills building on emerging epidemics, GBV, mental health, psychosocial support, student wellbeing, and other social challenges affecting students and communities
	Provide technical support to PSET institutions through the development and implementation of evidence-based guidelines, protocols, and coordinated response mechanisms for addressing health, psychosocial and community-centred challenges within the sector
5. To foster effective sectoral coordination and collaboration across the PSET system, ensuring the sharing of best practices and joint implementation	Establish and/or sustain national, provincial, and regional forums/communities of practice across universities, TVET colleges and CET colleges for regular knowledge-sharing, peer learning, and dissemination of best practices
	Institutionalise functioning health, wellness, and development committees (ICCs) at all PSET institutions to coordinate and monitor integrated student health, psychosocial, GBV, mental health, disability, and student development interventions
6. To mobilise sustainable national and global financial, technical, and strategic partnerships, underpinned by good governance and accountability, to strengthen holistic wraparound support	Establish and strengthen sustainable national partnerships with public, private, civil society and sectoral stakeholders
	Expand and sustain international partnerships and collaborations that contribute technical expertise, innovation, and resource mobilisation
	Mobilise diversified and sustainable funding and resource streams from national and global donors,

	development agencies, and government and private sector partners
	Strengthen organisational governance, accountability, compliance and sustainability through effective leadership, transparent financial management, risk management, and adherence to ethical and governance best practices

Risk identification and mitigation

As Higher Health embarks on the implementation of its five-year strategic plan, it recognises that achieving its vision, mission, goals and intended impact requires initiative-taking identification and management of strategic risks. Operating within a dynamic environment shaped by evolving social, economic, political, technological, and public health realities presents both opportunities and challenges that may influence organisational performance and long-term sustainability.

Higher Health is committed to keeping a robust risk management approach that enables the organisation to predict, assess and mitigate potential threats while strengthening resilience and organisational agility. The strategic risks outlined below represent key areas that may affect the successful delivery of our mandate, growth ambitions, partnerships, financial sustainability, operational effectiveness, and impact on beneficiaries.

By continuously monitoring these risks and implementing appropriate mitigation measures, Higher Health seeks to safeguard its mission, maximise opportunities for innovation and growth, and ensure the sustained delivery of high-quality programmes and services that improve the health, wellbeing, resilience and future prospects of young people and communities across South Africa.

The identified key strategic risks are presented below together with risk mitigation interventions.

1. Geopolitical Donor Shifts: Donor Funding Reduction and impact due to Geopolitical Instability

Mitigation

- ‡ Strategy approved by the Board
- ‡ Five-year Memorandum of Understanding (MoU) with DHET concluded
- ‡ Grant funding secured through the Skills Fund
- ‡ Curriculum developed to enable self-generated funding
- ‡ Business Development Unit set up with an anti-competition strategy in place

2. Working Capital Liquidity: Liquidity and Working Capital Depletion due to Higher Health growth, and Quick Higher Health Growth

Mitigation

- ‡ Cash flow monitoring workflows reviewed at EXCO and ARFIC levels.
- ‡ Financial Policies approved by the Board
- ‡ Cost Effective Measures in Managing systems -
- ‡ Cost containment and cost-cutting measures enforced.

3. Information Technology & Digital Sovereignty: Systemic Cyber Security Breaches & Identity Theft

Mitigation

- ‡ Cloud storage, multi-factor authentication (MFA)
- ‡ Active head-office firewalls
- ‡ Email security gateways
- ‡ ICT Policy and Strategy

4. Human Capital Sustainability: Executive Talent Deficit & Brain Drain

Mitigation

- ‡ Strong focus towards Succession planning
- ‡ Long fixed-term contracts
- ‡ Strengthening incentives that includes internal promotions, medical aid and retirement annuity allowances, along with provision of performance bonuses
- ‡ Recognition and rewards programme

5. Operational Performance & Data Integrity: Programme Target Shortfalls & Reporting Failures

Mitigation

- ‡ Weekly/Monthly monitoring system for early detection
- ‡ Integrated Digitalised data system for real-time reporting
- ‡ Developing data quality assurance system and framework

6. Statutory Governance & Compliance: Statutory Non-Compliance Penalties

Mitigation

- ‡ Monthly management committee meetings
- ‡ Compliance tracking
- ‡ Adherence to Company's Act; PBO; Accounting standards; SARS; LRA; PAJA; POPIA; BBBEE
- ‡ MOI and Board TOR

7. Systemic Financial Integrity (Fraud & Corruption): Procurement Fraud

Mitigation

- ‡ Whistleblowing and Hotlines
- ‡ Basic fraud awareness and training
- ‡ Compliance Review
- ‡ Delegation of Authority
- ‡ Approved Fraud Policy

8. Brand Dilution & Stakeholder Alienation: Public Relations & Social Media Backlash

Mitigation

- ‡ Inclusion of confidentiality social media compliance clauses in Service Level Agreements (SLAs) and employment contracts
- ‡ Legal, Communications, and Human Resources teams accountable for enforcement and oversight

A comprehensive table of risks appears in Annexure A.

Monitoring and evaluation

The monitoring and evaluation (M&E) framework feature a combination of impact and outcome indicators. The impact indicators are listed below while Annexure B holds a compact version of the full M&E matrix.

Higher Health is committed to measuring and showing meaningful impact in the lives of the individuals and communities it serves. As part of its five-year strategic framework, the organisation has shown a set of core impact indicators that will enable the systematic assessment of progress towards its vision, mission, and strategic goals.

These impact indicators will be measured through both longitudinal and real-time evaluation approaches. Longitudinal measurement will allow Higher Health to track sustained changes and outcomes experienced by beneficiaries over time, providing insight into the lasting impact of its interventions. Real-time measurement, through pre- and post-intervention assessments and continuous monitoring mechanisms, will enable the organisation to evaluate immediate changes in knowledge, attitudes, skills, behaviours, and wellbeing outcomes attributable to its programmes.

Together, these indicators will provide a robust evidence base to inform decision-making, strengthen programme effectiveness, ensure accountability to stakeholders, and show Higher Health's contribution to improving the health, wellbeing, resilience, employability, and life outcomes of the populations it serves.

M&E impact indicators

- † Number of learners enrolled on HIGHER HEALTH accredited skills programmes who attain an NQF Level 5 certificate
- † Percentage of HIGHER HEALTH accredited skills programme graduates who secure employment, self-employment, or further study within 12-24 months of completion
- † Percentage of HIGHER HEALTH accredited skills programme graduates who report sustained positive life-skills, civic and personal-development outcomes attributable to the programme
- † Percentage of learners completing accredited Civic and Soft Skills Programme who report both (a) acquiring relevant knowledge and (b) demonstrating skills acquisition and confidence (score 3 or 4 on ≥70% of topic items)
- † Percentage of HIGHER HEALTH accredited skills programme beneficiaries in PSET who report improved academic results during or following programme participation
- † Percentage of HIGHER HEALTH non-accredited skills programme graduates who report sustained positive life-skills, civic and personal-development outcomes attributable to the programme
- † Percentage of students disclosing a mental health concern who receive a timely response and are supported through the referral pathway to closure
- † Percentage change in reported Gender-Based Violence (GBV) cases within PSET institutions following HIGHER HEALTH interventions
- † Percentage of students with disabilities who report receiving the reasonable accommodation and wraparound support they need to participate fully in PSET

- ‡ Percentage of trained peer educators acquiring relevant knowledge, demonstrates skills acquisition and confidence to implement
- ‡ Percentage of trained frontline and managerial staff acquiring relevant knowledge, demonstrates skills acquisition and confidence to implement
- ‡ Number of PSET institutions with fully functional HICCS structures coordinating integrated student development

Refer to Annexure B for the M&E Strategic Indicator Matrix.